NETRAVATI GASTRO & CANCER HOSPITAL
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Reg. No. KA/DK/HOSP/2010/0412

COLONOSCOPY REPORT

Department of Medical Gastroenterology - Endoscopy Suite

Patient:	**RAGHAVENDRA V. SHETTY**	UHID:	**NGCH/25/88104**
Age / Sex:	59 Y / Male	Date:	**07/01/2026**
Endoscopist:	Dr. Ashwin Pai	Sedation:	Inj. Midazolam 2 mg + Inj. Fentanyl 50 mcg IV
Indication:	Bleeding per rectum x 3 months, altered bowel habit, weight loss 6 kg	Bowel prep:	PEG, adequate (Boston score 7)

Procedure

Digital rectal examination: hard, irregular growth felt at the fingertip, lower edge about 6 cm from the anal verge. Colonoscope passed up to the caecum; ileocaecal valve and appendicular orifice identified. Withdrawal time 9 minutes.

Findings

- Rectum: ulceroproliferative, friable, near-circumferential growth from 6 to 11 cm from the anal verge, narrowing the lumen; the scope could be negotiated beyond it with gentle pressure.
- Sigmoid colon: a few diverticula; no polyp.
- Rest of the colon up to the caecum: normal mucosa; no synchronous lesion.

Endoscopic diagnosis

Ulceroproliferative growth in the mid and lower rectum, suspicious of carcinoma.

Multiple biopsies (6 bits) taken from the growth and sent for HPE. The lower edge of the lesion tattooed with India ink in two quadrants.

Advice: serum CEA, MRI pelvis (rectal protocol) and CECT chest and abdomen. Review in Surgical Oncology OPD (Dr. Harish Bhandary) with the HPE report.

Dr. Ashwin Pai
M.D., D.M. (Gastroenterology)
Reg. No. KMC-71820
Consultant Gastroenterologist

SYNTHETIC DOCUMENT — FOR DEMONSTRATION ONLY